



PACKAGE INSERT
XALEXA 20

1. Product Name

XALEXA 20 (Paroxetine Tablets 20 mg)

2. Name and Strength of Active Ingredient (s)

Paroxetine Tablets 20 mg

3. Product Description

White to off-white coloured film –coated modified capsule shaped, biconvex tablets debossed with '56' on one side and 'C' with a deep break line on the other side. Each film-coated tablet contains: Paroxetine Hydrochloride Hemihydrate Ph. Eur. equivalent to Paroxetine 20 mg.

4. Pharmacodynamics / Pharmacokinetics
Pharmacodynamics

Paroxetine is a potent and selective inhibitor of 5-hydroxytryptamine (5-HT, serotonin) reuptake and its antidepressant action and efficacy in the treatment of OCD, Social Anxiety disorder / Social Phobia, General Anxiety Disorder, Post-traumatic Stress Disorder and panic disorder is thought to be related to its specific inhibition of 5-HT uptake in brain neurons.

Paroxetine is chemically unrelated to the tricyclic, tetracyclic and other available antidepressants. Paroxetine has low affinity for muscarinic cholinergic receptors and animal studies have indicated only weak anti-cholinergic properties. Paroxetine does not impair psychomotor function and does not potentiate the depressant effects of ethanol.

In the treatment of depressive disorders, Paroxetine exhibits comparable efficacy to standard antidepressants.

Morning dosing with Paroxetine does not have any detrimental effect on either the quality or duration of sleep. Moreover, patients are likely to experience improved sleep as they respond to Paroxetine therapy.

Dose response:

In the fixed doses there is a flat dose response curve, providing no suggestion of advantage in terms of efficacy for using higher than the recommended doses.

Long-term efficacy:

The long-term efficacy of Paroxetine in treating social anxiety disorder and generalised anxiety disorder and Post-traumatic Stress Disorder has not been sufficiently demonstrated.

Pharmacokinetics

Absorption:

Paroxetine is well absorbed after oral dosing and undergoes first-pass metabolism. Due to first-pass metabolism, the amount of Paroxetine available to the systemic circulation is less than that absorbed from the gastrointestinal tract. Partial saturation of the first-pass effect and reduced plasma clearance occur as the body burden increases with higher single doses or on multiple dosing. This results in disproportionate increases in plasma concentrations of Paroxetine and hence pharmacokinetic parameters are not constant, resulting in non-linear kinetics. However, the non-linearity is generally small and is confined to those subjects who achieve low plasma levels at low doses.

Steady state systemic levels are attained by 7 to 14 days after starting treatment with immediate or controlled release formulations and pharmacokinetics do not appear to change during long-term therapy.

Distribution:

Paroxetine is extensively distributed into tissues and Pharmacokinetic calculations indicate that only 1% of the Paroxetine in the body resides in the plasma. Approximately 95% of the Paroxetine present is protein bound at therapeutic concentrations.

Metabolism:

The principal metabolites of Paroxetine are polar and conjugated products of oxidation and methylation which are readily cleared. In view of their relative lack of pharmacological activity, it is most unlikely that they contribute to Paroxetine therapeutic effects. Metabolism does not compromise Paroxetine selective action on neuronal 5-HT uptake.

Elimination:

Urinary excretion of unchanged Paroxetine is generally less than 2% of dose whilst that of metabolites is about 64% of dose. About 36% of the dose is excreted in faeces, probably via the bile, of which unchanged Paroxetine represents less than 1 % of the dose. Thus Paroxetine is eliminated almost entirely by metabolism. Metabolite excretion is biphasic, being initially a result of first-pass metabolism and subsequently controlled by systemic elimination of Paroxetine.

The elimination half-life is variable but is generally about 1 day.

5. Indication

Adults Depression:

Treatment of symptoms of depressive illness of all types including reactive and severe depression and depression accompanied by anxiety. Following an initial satisfactory response, continuation with XALEXA therapy is effective in preventing relapse of Depression.

Anxiety Disorders: Treatment of symptoms and prevention of relapse of Obsessive Compulsive Disorder (OCD). Treatment of symptoms and prevention of relapse of Panic Disorder with or without agoraphobia. Treatment of Social Anxiety Disorder/Social Phobia. Treatment of symptoms and prevention of relapse of Generalised Anxiety Disorder. Treatment of Post-traumatic Stress Disorder.

Children and adolescents (less than 18 years):

All Indications: XALEXA is not indicated for use in children or adolescents aged less than 18 years (see Warnings and Precautions). Controlled clinical studies in children and adolescents with major depressive disorder failed to demonstrate efficacy, and do not support the use of XALEXA in the treatment of depression in this population (see Warnings and Precautions). The safety and efficacy of XALEXA in children aged less than 7 years has not been studied.'

6. Posology and Mode of administration
Adults:

It is recommended that XALEXA be administered once daily in the morning with food. The tablets should be swallowed rather than chewed. As with all antidepressant drugs, dosage should be reviewed and adjusted if necessary within two to three weeks of initiation of therapy and thereafter as judged clinically appropriate. Patients should be treated for a sufficient period to ensure that they are free from symptoms. This period may be several months for depression and may be even longer for OCD and panic disorder. As with many psychoactive medications, abrupt discontinuation should be avoided (see Adverse Reactions).

Depression:

The recommended dose is 20 mg daily. In some patients it may be necessary to increase the dose. This should be done gradually by 10 mg increments to a maximum of 50 mg according to the patient's response.

Obsessive Compulsive Disorder:

The recommended dose is 40 mg daily. Patients should start on 20 mg daily and the dose may be increased weekly in 10 mg increments. Some patients will benefit from having their dose increased up to a maximum of 60 mg daily.

Panic Disorder:

The recommended dose is 40 mg daily. Patients should be started on 10 mg daily and the dose increased weekly in 10 mg increments according to the patient's response. Some patients may benefit from having their dose increased up to a maximum of 50 mg daily. As is generally recognised, there is the potential for worsening of panic symptomatology during early treatment of panic disorder; a low initial starting dose is therefore recommended.

Social Anxiety Disorder/Social Phobia:

The recommended dose is 20 mg daily. Patients not responding to a 20 mg dose may benefit from dose increases in 10 mg increments as required, up to a maximum of 50 mg/day. Dose changes should occur at intervals of at least 1 week.

Generalised Anxiety Disorder:

The recommended dose is 20 mg daily. Some patients not responding to a 20 mg dose may benefit from having dose increases in 10 mg increments as required, up to a maximum of 50 mg/day according to patient's response.

Post-traumatic Stress Disorder: The recommended dose is 20 mg daily. Some patients not responding to a 20 mg dose may benefit from having dose increases in 10 mg increments as required, up to a maximum of 50 mg/day according to the patient's response.

General Information Discontinuation of XALEXA
As with other psychoactive medications, abrupt discontinuation should generally be avoided (see Warnings and Precautions for Use & Adverse Reactions). The taper phase regimen used in recent clinical trials involved a decrease in the daily dose by 10 mg/day at weekly intervals. When a daily dose of 20 mg/day was reached, patients were continued on this dose for one week before treatment was stopped. If intolerable symptoms occur following a decrease in the dose or upon discontinuation of treatment, then resuming the previously prescribed dose may be considered. Subsequently, the physician may continue decreasing the dose, but at a more gradual rate.

Other populations:

Elderly: Increased plasma concentrations of paroxetine occur in elderly subjects. Dosing should commence at the adult starting dose and may be increased weekly in 10 mg increments to a maximum of 40 mg daily according to the patient's response.

Children and adolescents (less than 18 years):

XALEXA is not indicated for use in children or adolescents aged less than 18 years (see Indications and Warnings and Precautions).

Renal/hepatic impairment: Increased plasma concentrations of paroxetine occur in patients with severe renal impairment (creatinine clearance <30 ml/min) or in those with hepatic impairment. The recommended dose is 20 mg daily. Incremental dosage, if required, should be restricted to the lower end of the range.

7. Contraindication

Known hypersensitivity to paroxetine and excipients of XALEXA.

XALEXA should not use in combination with monoamine oxidase (MAO) inhibitors or within 2 weeks of terminating treatment with MAO inhibitors. Likewise, MAO inhibitor should not be introduced within 2 weeks of cessation of therapy with XALEXA.

XALEXA should not use in combination with thioridazine, because as with other drugs which inhibit the hepatic enzymes CYP-450 2D6, paroxetine can elevate plasma levels of thioridazine.

Administration of thioridazine alone can lead to QTc interval prolongation with associated serious ventricular arrhythmia eg, Torsades de pointes and sudden death.

8. Warnings and Precautions

Clinical worsening and suicide risk in adult:

Young adults, especially those with major depressive disorder (MDD), may be at increased risk for suicidal behaviour during treatment with XALEXA. Patients with depression may experience worsening of their depressive symptoms and/or the emergence of suicidal ideation and behaviours (suicidality) whether or not they are taking antidepressant medications. The risk persists until significant remission occurs. It is general clinical experience with all antidepressant therapies that the risk of suicide may increase in the early stages of recovery. Other psychiatric conditions for which XALEXA is prescribed can be associated with an increased risk of suicidal behaviour and these conditions may also be co-morbid with MDD. Additionally, patients with a history of suicidal behaviour or thoughts, young adults and those patients exhibiting a significant degree of suicidal ideation prior to commencement of treatment are at a greater risk of suicidal thoughts or suicide attempts. All patients should be monitored for clinical worsening (including development of new symptoms) and suicidality throughout treatment, and especially at the beginning of a course of treatment, or at the time of dose changes, either increases or decreases.

Patients (and caregivers of patients) should be alerted about the need to monitor for any worsening of their condition (including development of new symptoms) and/or the emergence of suicidal ideation/behaviour or thoughts of harming themselves and to seek medical advice immediately if these symptoms present. It should be recognised that the onset of some symptoms eg, agitation, akathisia, or mania, could be related either to the underlying disease state or the drug therapy (see Akathisia and Mania and Bipolar Disorder as follow and adverse Reaction).

Consideration should be given to changing the therapeutics regimen, including possibly discontinuing XALEXA, in patients who experience clinical worsening (including development of new symptoms) and/or the emergence of suicidal ideation/behaviour, especially if these symptoms are severe, abrupt in onset, or were not part of the patient's presenting symptoms.

Akathisia: Rarely, the use of XALEXA has been associated with the development of akathisia, which characterised by an inner sense of restlessness, and psychomotor agitation eg. An inability to sit or stand still usually associated with subjective distress. This is most likely to occur within the first few weeks of treatment.

Serotonin Syndrome/ Neuroleptic Malignant Syndrome:

On rare occasions, development of a serotonin syndrome or neuroleptic malignant syndrome like events may occur in association with XALEXA treatment, particularly when given in combination with other serotonergic and/or neuroleptics drugs. As these syndromes may result in potentially life-threatening conditions, treatment with XALEXA should be discontinued if such events occur and supportive symptomatic treatment should be used in combination with serotonin precursors (eg, L-tryptophan, oxitriptan) due to the risk of serotonin syndrome.

Mania and Bipolar Disorder: A major depressive episode may be the initial presentation of bipolar disorder. It is generally believed (though not established in controlled trials) that treating such an episode with an antidepressant alone can increase the likelihood of precipitation of a mixed/manic episode in patients at risk for bipolar disorder. Prior to initiating treatment with an antidepressant, patients should be adequately screened to determine if they are at risk for bipolar disorder; such screening should include a detailed psychiatric history, including a family history of suicide, bipolar disorder and depression. It should be noted that paroxetine is not approved for use in treating bipolar depression. As with all antidepressants, paroxetine should be used with caution in patients with a history of mania.

Monoamine Oxidase Inhibitors: Treatment with XALEXA should be initiated cautiously at least 2 weeks after terminating treatment with MAO inhibitors and dosage of XALEXA should be increased gradually until optimal response is reached.

■ Black A/s: 110 x 470 mm

 AUROBINDO Packaging Development		Product Name	Component	Item Code	Date & Time
		Xalexa Tablets	Leaflet	P1520917	11.03.2019 & 12.10 PM
Team LeaderKiran InitiatorC. Viswanath		Customer / Country	Version No.	Reason Of Issue	Reviewed / Approved by
		Malaysia	01	NEW	
Artist:		Dimensions	No. of Colours : 01		
		110 x 470 mm			
Additional Information :		Pharmacode			
		20917			
					Sign / Date

P1520917

Renal/Hepatic Impairment: Caution is recommended in patients with severe renal impairment or in those with hepatic impairment.

Epilepsy: as with other antidepressants, XALEXA should be used with caution in patients with epilepsy. Seizures: Overall, the incidence of seizures is <0.1% in patients treated with paroxetine. XALEXA should be discontinued in any patient who develops seizures.

Electroconvulsive Therapy (ECT): There is little clinical experience of the concurrent administration of XALEXA with ECT.

Glaucoma: As with other SSRI's, paroxetine infrequently causes mydriasis and should be used with caution in patients with narrow-angle glaucoma.

Hyponatraemia: Hyponatraemia has been reported rarely, predominantly in the elderly. The hyponatraemia generally reverses on discontinuation of paroxetine.

Haemorrhage: Skin and mucous membrane bleedings (including gastrointestinal bleeding) have been reported following treatment with paroxetine. Paroxetine should therefore be used with caution in patients concomitantly treated with drug that given an increased risk for bleeding, and in patients with known tendency for bleeding or those with predisposing condition.

Cardiac condition: the usual precautions should be observed in patients with cardiac condition.

Dizziness, sensory disturbance (including paraesthesia, electric shock sensations and tinnitus), sleep disturbance (including intense dreams), agitation or anxiety, nausea, tremor, confusion, sweating, headache, diarrhoea have been reported. Generally, these symptoms are mild to moderate; however, in some patients they may be severe in intensity. They usually occur within the first few days of discontinuing treatment, but there have been very rare reports of such symptoms in patients who have in advertently missed a dose. Generally, these symptoms are self-limiting and usually resolve within 2 weeks, though in some individuals they may be prolonged (≥2-3 months). It is therefore advised that XALEXA should be gradually tapered when discontinuing treatment over a period of several weeks or months, according to patient's need.

9. Interactions with other medicaments
Serotonergic medicinal products:

As with other SSRIs, co-administration with serotonergic medicinal products may lead to an incidence of 5-HT associated effects. Caution should be advised and a closer clinical monitoring is required when serotonergic drugs (such as L-tryptophan, triptans, tramadol, linezolid, methylthionium chloride (methylene blue), SSRIs, lithium, pethidine and St. John's Wort – Hypericum perforatum – preparations) are combined with paroxetine. Caution is also advised with fentanyl used in general anaesthesia or in the treatment of chronic pain.

Pravastatin: An interaction between paroxetine and pravastatin has been observed in studies suggesting that co-administration of paroxetine and pravastatin may lead to an increase in blood glucose levels. Patients with diabetes mellitus receiving both paroxetine and pravastatin may require dosage adjustment of oral hypoglycaemic agents and/or insulin.

Pimozide: Increased pimozide levels of on average 2.5 times have been demonstrated in a study of a single low dose pimozide (2 mg) when co-administered with 60 mg paroxetine. This may be explained by the known CYP2D6 inhibitory properties of paroxetine, due to the narrow therapeutic index of pimozide and its known ability to prolong QT interval, concomitant use of pimozide and paroxetine is contraindicated.

Drug metabolizing enzymes: The metabolism and pharmacokinetics of paroxetine may be affected by the induction or inhibition of drug metabolizing enzymes. When paroxetine is to be co-administered with a known drug metabolising enzyme inhibitor, consideration should be given to using paroxetine doses at the lower end of the range. No initial dosage adjustment is considered necessary when the drug is to be co-administered with known drug metabolising enzyme inducers (e.g. carbamazepine, rifampicin, phenobarbital, phenytoin)) or with fosamprenavir/ritonavir. Any paroxetine dosage adjustment (either after initiation or following discontinuation of an enzyme inducer) should be guided by clinical effect (tolerability and efficacy).

Fosamprenavir/ritonavir: Co-administration of fosamprenavir/ritonavir 700/100 mg twice daily with paroxetine 20 mg daily in healthy volunteers for 10 days significantly decreased plasma levels of paroxetine by approximately 55%. The plasma levels of fosamprenavir/ritonavir during co-administration of paroxetine were similar to reference values of other studies, indicating that paroxetine had no significant effect on metabolism of fosamprenavir/ritonavir. There are no data available about the effects of long-term co-administration of paroxetine and fosamprenavir/ritonavir exceeding 10 days.'

Procyclidine: Daily administration of paroxetine increases significantly the plasma levels of procyclidine. If anti-cholinergic effects are seen, the dose of procyclidine should be reduced. Anticonvulsants: Carbamazepine, phenytoin, sodium valproate. Concomitant administration does not seem to show any effect on pharmacokinetic/dynamic profile in epileptic patients.

CYP2D6 inhibitory potency of paroxetine: As with other antidepressants, including other SSRIs, paroxetine inhibits the hepatic cytochrome P450 enzyme CYP2D6. Inhibition of CYP2D6 may lead to increased plasma concentrations of co-administered medicinal products metabolized by this enzyme. These include certain tricyclic antidepressants (e.g. clomipramine, nortriptyline and desipramine), phenothiazine neuroleptics (e.g. perphenazine and thioridazine.), risperidone, atomoxetine, certain Type 1c antiarrhythmics (e.g. propafenone and flecainide) and metoprolol. It is not recommended to use paroxetine in combination with metoprolol when given in cardiac insufficiency, because of the narrow therapeutic index of metoprolol in this indication. Pharmacokinetic interaction between CYP2D6 inhibitors and tamoxifen, showing a 65-75% reduction in plasma levels of one of the more active forms of tamoxifen, i.e. endoxifen, has been reported in the literature. Reduced efficacy of tamoxifen has been reported with concomitant usage of some SSRI antidepressants in some studies. As a reduced effect of tamoxifen cannot be excluded, co-administration with potent CYP2D6 inhibitors (including paroxetine) should whenever possible be avoided.

Alcohol: As with other psychotropic medicinal products patients should be advised to avoid alcohol use while taking paroxetine.

Oral anticoagulants:

A pharmacodynamics interaction between paroxetine and oral anticoagulants may occur. Concomitant use of paroxetine and oral anticoagulants can lead to an increased anticoagulant activity and haemorrhagic risk. Therefore, paroxetine should be used with caution in patients who are treated with oral anticoagulants.

NSAIDs and acetylsalicylic acid, and other antiplatelet agents:

A pharmacodynamic interaction between paroxetine and NSAIDs/acetylsalicylic acid may occur. Concomitant use of paroxetine and NSAIDs/acetylsalicylic acid can lead to an increased haemorrhagic risk. Caution is advised

in patients taking SSRIs, concomitantly with oral anticoagulants, medicinal products known to affect platelet function or increase risk of bleeding (e.g. atypical antipsychotics such as clozapine, phenothiazines, most TCAs, acetylsalicylic acid, NSAIDs, COX-2 inhibitors) as well as in patients with a history of bleeding disorders or conditions which may predispose to bleeding.

10. Pregnancy and Lactation
Pregnancy

Some epidemiological studies suggest an increased risk of congenital malformations, particularly cardiovascular (e.g. ventricular and atrial septum defects) associated with the use of paroxetine during the first trimester. The mechanism is unknown. The data suggests that the risk of having an infant with a cardiovascular defect following maternal paroxetine exposure is less than 2/100 compared with an expected rate for such defects of approximately 1/100 in the general population.

Paroxetine should only be used during pregnancy when strictly indicated. The prescribing physician will need to weigh the option of alternative treatments in women who are pregnant or are planning to become pregnant.

Abrupt discontinuation should be avoided during pregnancy. Neonates should be observed if maternal use of paroxetine continues into the later stages of pregnancy, particularly the third trimester.

The following symptoms may occur in the neonates after maternal paroxetine use in later stages of pregnancy: respiratory distress, cyanosis, apnoea, seizures, temperature instability, feeding difficulty, vomiting, hypoglycaemia, hypertonia, hypotonia, hyperreflexia, tremor, jitteriness, irritability, lethargy, constant crying, somnolence and difficulty in sleeping. These symptoms could be due to either serotonergic effects or withdrawal symptoms. In a majority of instances the complications begin immediately or soon (<24 hours) after delivery.

Epidemiological data have suggested that the use of SSRIs in pregnancy, particularly in late pregnancy, may have an increased risk of persistent pulmonary hypertension of the newborn (PPHN). The observed risk was approximately 5 cases per 1000 pregnancies. In the general population 1 to 2 cases of PPHN per 1000 pregnancies occur.

Breast-feeding

Small amounts of paroxetine are excreted into breast milk. Serum concentrations in breast-fed infants were undetectable (<2 ng/ml) or very low (<4 ng/ml) and no signs of medicinal product effects were observed in these infants. Since no effects are anticipated, breast-feeding can be considered.

11. Undesirable Effects

Some of the adverse drug reactions listed below may decrease in intensity and frequency with continued treatment and do not generally lead to cessation of therapy. Adverse drug reactions are listed below by system organ class and frequency

Blood and lymphatic system disorders: Abnormal bleeding, predominantly of the skin and mucous membranes (mostly ecchymosis).

Endocrine disorders: syndrome of inappropriate anti-diuretic hormone secretion (SIADH).

Metabolism and nutrition disorders: Decreased appetite, increases in cholesterol levels.

Psychiatric disorders: Somnolence (sleepiness), insomnia (sleeping difficulties), agitation, confusion, hallucinations (hearing voices or seeing things which are not there), manic reactions (unusual behaviour caused by uncontrollable excitement), anxiety.

Nervous system disorders: dizziness, tremor, convulsions, serotonin syndrome (symptoms may include agitation, confusion, diaphoresis (uncontrolled excessive sweating), hallucinations, hyperreflexia (over responsive reflexes), myoclonus (muscle twitching), shivering, tachycardia (rapid heartbeat) and tremor)

Eye disorders: Blurred vision and acute glaucoma

Vascular disorders: Transient increases or decreases in blood pressure, postural hypotension

Gastrointestinal disorders: nausea, constipation, diarrhoea, vomiting, dry mouth.

Hepato-biliary disorders: Elevation of hepatic enzymes, liver events (such as hepatitis, sometimes associated with jaundice and/or liver failure).

Skin and subcutaneous tissue disorders: Sweating, skin rashes, pruritus.

Musculoskeletal and connective tissue disorders: arthralgia, myalgia.

Renal and urinary disorders: urinary retention, urinary incontinence

Reproductive system and breast disorders: sexual dysfunction.

General disorders and administration site conditions: asthenia, body weight gain, peripheral oedema.

Withdrawal symptoms seen on discontinuation of paroxetine treatment: dizziness, sensory disturbances, sleep disturbances, anxiety, headache, agitation, nausea, tremor, confusion, sweating, emotional instability, visual disturbances, palpitations, diarrhoea, and irritability.

General disorders and administration site conditions: asthenia (weakness), body weight gain, peripheral oedema (fluid retention). Other side effects not listed above may also occur in some people.

12. Overdose

Symptoms and signs

A wide margin of safety is evident from available overdose information on paroxetine. Experience of paroxetine in overdose has indicated that, in addition to those symptoms mentioned such as vomiting, dilated pupils, fever, blood pressure changes, headache, involuntary muscle contractions, agitation, anxiety and tachycardia was experienced. Patients have generally recovered without serious sequel even when doses of up to 2000 mg have been taken alone. Events such as coma or ECG changes have occasionally and, very rarely with a fatal outcome, but generally when paroxetine was taken in conjunction with other psychotropic medicinal products, with or without alcohol.

Treatment

No specific antidote is known.

The treatment should consist of those general measures employed in the management of overdose with any antidepressant. Administration of 20-30 g activated charcoal may be considered if possible within a few hours after overdose intake to decrease absorption of paroxetine. Supportive care with frequent monitoring of vital signs and careful observation is indicated. Patient management should be as clinically indicated.

13. Storage Condition

Do not store above 30 °C.

14. Shelf Life

48 months

15. Dosage Forms and Packaging available

3 X 10'S of aluminium blister pack with 20mg Paroxetine tablets.

16. Name and address of manufacturer

Aurobindo Pharma Limited (Unit-III),
Sy. No 313 & 314 Bachupally (V), Bachupally
Mandal, Medchal-Malkajigiri District, Telangana State,.,
India

17. Product registration holder

UNIMED SDN BHD, 53, Jalan Tembaga SD 5/2b,
Bandar Sri Damansara, 52200 Kuala Lumpur,
Selangor, Malaysia

18. Date of revision 27/10/2023